

In 1965, Stuart Oskamp supplied clinical psychologists with successive files, each containing an increasing amount of information about patients; the psychologists' diagnostic abilities did not grow with the additional supply of information. They just got more confident in their original diagnosis. Granted, one may not expect too much of psychologists of the 1965 variety, but these findings seem to hold across disciplines.

Finally, in another telling experiment, the psychologist Paul Slovic asked bookmakers to select from eighty-eight variables in past horse races those that they found useful in computing the odds. These variables included all manner of statistical information about past performances. The bookmakers were given the ten most useful variables, then asked to predict the outcome of races. Then they were given ten more and asked to predict again. The increase in the information set did not lead to an increase in their accuracy; their confidence in their choices, on the other hand, went up markedly. Information proved to be toxic. I've struggled much of my life with the common middlebrow belief that "more is better"—more is sometimes, but not always, better. This toxicity of knowledge will show in our investigation of the so-called expert.

THE EXPERT PROBLEM, OR THE TRAGEDY OF THE EMPTY SUIT

So far we have not questioned the authority of the professionals involved but rather their ability to gauge the boundaries of their own knowledge. Epistemic arrogance does not preclude skills. A plumber will almost always know more about plumbing than a stubborn essayist and mathematical trader. A hernia surgeon will rarely know less about hernias than a belly dancer. But their probabilities, on the other hand, will be off—and, this is the disturbing point, you may know much more on that score than the expert. No matter what anyone tells you, it is a good idea to question *the error rate* of an expert's procedure. Do not question his procedure, only his confidence. (As someone who was burned by the medical establishment, I learned to be cautious, and I urge everyone to be: if you walk into a doctor's office with a symptom, do not listen to his odds of its *not* being cancer.)

I will separate the two cases as follows. The mild case: *arrogance in the presence of (some) competence*, and the severe case: *arrogance mixed with incompetence (the empty suit)*. There are some professions in which you know more than the experts, who are, alas, people for whose opinions you are paying—instead of them paying you to listen to them. Which ones?

What Moves and What Does Not Move